

Pharmacode position may change as per Supplier’s m/c requirement & additional small pharma code may appear on the front / back panel



Package Insert
SARANTO 50
(Losartan Potassium Tablets 50 mg)
Rx Only

NAME OF THE PRODUCT:
SARANTO 50 (Losartan Potassium Tablets 50 mg)
NAME AND STRENGTH OF ACTIVE INGREDIENT:
Losartan Potassium Ph.Eur. and 50 mg.

DOSAGE FORM: Tablet.
PRODUCT DESCRIPTION:
White to off-white, oval shaped, biconvex film coated tablets debossed with ‘E’ on one side and ‘4’ and ‘6’ separated by a score line on other side.

Pharmacodynamics/Pharmacokinetics
Pharmacodynamics:
Losartan is an oral, specific angiotensin-II receptor (type AT1) antagonist. Angiotensin II binds to the AT1 receptor found in many tissues (e.g. vascular smooth muscle, adrenal gland, kidneys, and the heart) and elicits several important biological actions, including vasoconstriction and the release of aldosterone. Angiotensin II also stimulates smooth-muscle proliferation. Based on binding and pharmacological bioassays, it binds selectively to the AT1 receptor.

During Losartan administration, removal of angiotensin-II negative feedback on renin secretion leads to increased plasma renin activity. Increases in plasma renin activity lead to increases in angiotensin II in plasma. Even with these increases, antihypertensive activity and suppression of plasma aldosterone concentration are maintained, indicating effective angiotensin-II receptor blockade.

Losartan binds selectively to the AT1 receptor and does not bind to or block other hormone receptors or ion channels important in cardiovascular regulation. Furthermore, Losartan does not inhibit ACE (kininase II), the enzyme that degrades bradykinin. Losartan block responses to angiotensin I and angiotensin II without affecting responses to bradykinin.

In non-diabetic hypertensive patients with proteinuria, the administration of Losartan potassium significantly reduces proteinuria, fractional excretion of albumin and IgG. Losartan maintains glomerular filtration rate and reduces filtration fraction. Generally, Losartan causes a decrease in serum uric acid (usually < 24 micromole) which was persistent in chronic therapy.

Losartan has no effect on autonomic reflexes and no sustained effect on plasma noradrenaline. Losartan potassium administered in doses of up to 150 mg once daily did not cause clinically important changes in fasting triglycerides, total cholesterol or HDL cholesterol in patients with hypertension.

Although Losartan Potassium is antihypertensive in all races, as with other drugs that affect the renin angiotensin – aldosterone system, black hypertensive patients have a smaller average response to Losartan monotherapy than non-black patients.

Pharmacokinetic properties
Absorption

Following oral administration, Losartan is well absorbed and undergoes first-pass metabolism, forming an active carboxylic acid metabolite and other inactive metabolites. The systemic bioavailability of Losartan tablets is approximately 33%. Mean peak concentrations of Losartan and its active metabolite are reached in 1 hour and in 3-4 hours, respectively.

Distribution
Both Losartan and its active metabolite are 99% bound to plasma proteins, primarily albumin. The volume of distribution of Losartan is 34 litres.

Biotransformation
About 14% of an intravenously or orally administered dose of Losartan is converted to its active metabolite. Following oral and intravenous administration of ¹⁴C-labelled Losartan Potassium, circulating plasma radioactivity primarily is attributed to Losartan and its active metabolite. In addition to the active metabolite, inactive metabolites are formed, including two major metabolites formed by hydroxylation of the butyl side chain and a minor metabolite, an N-2 tetrazole glucuronide.

Elimination
Plasma clearance of Losartan and its active metabolite is about 600 ml/min and 50 ml/min, respectively. Renal clearance of Losartan and its active metabolite is about 74 ml/min and 26 ml/min, respectively. When Losartan is administered orally, about 4% of the dose is excreted unchanged in the urine, and about 6% of the dose is excreted in the urine as active metabolite. The pharmacokinetics of Losartan and its active metabolite are linear with oral Losartan Potassium doses up to 200 mg.

Following oral administration, plasma concentrations of Losartan and its active metabolite decline polyexponentially with a terminal half-life of about 2 hours and 6-9 hours, respectively. During once daily dosing with 100 mg, neither Losartan nor its active metabolite accumulates significantly in plasma.

Both biliary and urinary excretion contributes to the elimination of Losartan and its metabolites. Following an oral dose of ¹⁴C-labelled Losartan in man, about 35% of radioactivity is recovered in the urine and 58% in the faeces.

Indication
Hypertension
Losartan Potassium is indicated for the treatment of hypertension.

Hypertensive patients with Left Ventricular Hypertrophy
Losartan Potassium is indicated to reduce the risk of stroke in patients with hypertension and left ventricular hypertrophy.

Nephropathy in Type 2 Diabetic Patients
Indicated for the treatment of diabetic nephropathy with an elevated serum creatinine and proteinuria (urinary albumin to creatinine ratio ≥ 300 mg/g) in patients with type 2 diabetes and a history of hypertension. In this population Losartan Potassium reduces the rate of progression of neuropathy as measured by the occurrence of doubling the serum creatinine or end stage renal disease (need for dialysis or renal transplantation) or death.

Recommended Dose
The starting and maintenance dose is 50 mg once daily for most patients.

Mode of Administration
Losartan Potassium may be administered with or without food.

Losartan Potassium may be administered with other antihypertensive agents.

Hypertension
The usual starting and maintenance dose is 50 mg once daily for most patients. The maximal antihypertensive effect is attained 3-6 weeks after initiation of therapy. Some patients may receive an additional benefit by increasing the dose to 100 mg once daily.

For patients with intravascular volume depletion (eg., those treated with high dose diuretics), a starting dose of 25 mg once daily should be considered. (see Precautions)

No initial dosage adjustment is necessary for elderly patients or for patients with renal impairment, including patients on dialysis. A lower dose should be considered for patients with a history of hepatic impairment. (see Precautions)

Hypertensive Patients with Left Ventricular Hypertrophy
The usual starting dose is 50 mg of Losartan Potassium tablets once daily. A low dose of hydrochlorothiazide should be added and/or the dose of Losartan potassium should be increased to 100 mg once daily based on blood pressure response.

Nephropathy in Type II Diabetic Patients
The usual starting dose is 50 mg once daily. The dose may be increased to 100 mg daily based on blood pressure response. Losartan Potassium may be administered with other antihypertensive agents (e.g. diuretics, calcium channel blockers, alpha – or beta –blockers, and centrally acting agents) as well as with insulin and other commonly used hypoglycaemic agents (e.g., sulfonylureas, glitazones and glucosidase inhibitors)

Contraindication
Concomitant use of (active ingredient of ARB) with aliskiren-containing products is contraindicated in patients with diabetes mellitus or renal impairment (GFR < 60 ml/min/1.73 m²)
Losartan Potassium is contraindicated in pregnancy and in patients who are hypersensitive to any component of this product.

Warning and Precautions
Hypersensitivity:
Angioedema.
Losartan Potassium in patients with haemodynamically significant obstructive valvular disease or cardiomyopathy.

Hypotension and electrolyte/fluid im-balance
In patients who are intravascularly volume depleted (e.g. those treated with high-dose diuretics), symptomatic hypotension may occur. These conditions should be corrected prior to administration of Losartan Potassium or a lower starting dose should be used. In patients who are intravascularly volume-depleted, these conditions should be corrected prior to administration of Losartan Potassium. Electrolyte imbalances are common in patients with renal impairment, with or without diabetes, and should be addressed.

Liver function impairment
Based on pharmacokinetic data which demonstrate significantly increased plasma concentrations of losartan in cirrhotic patients, a lower dose should be considered for patients with a history of hepatic impairment

Renal function impairment
As a consequence of inhibiting the renin angiotensin system, changes in renal function including renal failure have been reported in susceptible individuals; these changes in renal function may be reversible upon discontinuation of therapy.

Other drugs that effect the renin-angiotensin system may increase blood urea and serum creatinine in patients with bilateral renal artery stenosis or stenosis of the artery to a solitary kidney.

Race (Black patients):
Losartan Potassium may reduce the risk of stroke in black patients with hypertension and left ventricular hypertrophy.

Interaction with other Medicaments
ACE inhibitors: The use of (active ingredient of ARB) with an ACE inhibitor may increase the risk of hyperkalaemia, hypotension, and syncope, particularly in patients with athero-

A/s: 210 x 300 mm ■ Black

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		SARANTO	Leaflet	P1538475	17.12.2024 & 04:00 pm
		Country	Version No.	Reason Of Issue	Reviewed / Approved by
		Malaysia - U3	00	Submission	
Team Leader	Kiran	Dimensions (mm)	Colours: 01		
Initiator	Shirisha	A/s:210 x 300 mm	 38475		
Artist	Advnt - Anji	Pharma Code: 38475			
Additional Information: Supersede code: P1514417					

sclerotic disease or heart failure, or in diabetics who have end–organ damage. Such combinations should be reserved for selected cases with close monitoring of renal function.

No drug interactions occur with hydrochlorothiazide, digoxin, warfarin, cimetidine, ketoconazole, erythromycin and phenobarbital (phenobarbitone). Rifampicin and fluconazole reduce levels of active metabolite.

As with other drugs that block angiotensin II or its effects, concomitant use of potassium-sparing diuretics (e.g., spironolactone, triamterene, amiloride), potassium supplements, or salt substitutes containing potassium may lead to increases in serum potassium.

As with other drugs which affect the excretion of sodium, lithium excretion may be reduced. Therefore, serum lithium levels should be monitored carefully if lithium salts are to be co-administered with angiotensin II receptor antagonists.

Non-steroidal anti-inflammatory drugs (NSAIDs) including selective cyclooxygenase-2 inhibitors (COX-2 inhibitors) may reduce the effect of diuretics and other antihypertensive drugs. Therefore, the antihypertensive effect of angiotensin II receptor antagonists may be attenuated by NSAIDs including selective COX-2 inhibitors.

In some patients with compromised renal function who are being treated with non-steroidal anti-inflammatory drugs, including selective cyclooxygenase-2 inhibitors, the co-administration of angiotensin II receptor antagonists may result in a further deterioration of renal function. These effects are usually reversible.

Pregnancy and Lactation

Pregnancy

When used pregnancy during the second and third trimesters, drugs that act directly on the renin angiotensin system can cause injury and even death in the developing foetus. When pregnancy is detected, Losartan Potassium Tablets should be discontinued as soon as possible.

Lactation

It is not known whether losartan is excreted in human milk. Because many drugs are excreted in human milk and because of the potential for adverse effects on the nursing infant, a decision should be made whether to discontinue nursing or discontinue the drug, taking into account the importance of the drug to the mother.

Side Effects

Dizziness may occur. In addition, dose-related orthostatic effects may be seen.

The most common drug-related side effects were dizziness, asthenia/fatigue and vertigo. Losartan Potassium was generally well tolerated in type 2 diabetic patients with nephropathy. The most common drug-related side effects were asthenia/fatigue, dizziness, hypotension and hyperkalaemia.

The following adverse reactions may occur:

Hypersensitivity: Anaphylactic reactions, angioedema including swelling of the larynx and glottis causing airway obstruction and/or swelling of the face, lips, pharynx, and/or tongue occur rarely in patients treated with Losartan; some of these patients previously experienced angioedema with other drugs including ACE inhibitors. Vasculitis, including Henoch-Schonlein purpura, may occur rarely.

Gastro-intestinal: Hepatitis (rarely), diarrhoea, and liver function abnormalities.

Haematologic: Anaemia, thrombocytopenia.

Musculoskeletal: Myalgia, arthralgia.

Nervous system/Psychiatric: Migraine.

Respiratory: Cough.

Skin: Urticarial, pruritus, rash.

Symptoms and Treatment of Overdose

The most likely manifestation of overdosage would be hypotension and tachycardia; bradycardia could occur from parasympathetic (vagal) stimulation. If symptomatic hypotension may occur, supportive treatment should be instituted. Neither Losartan nor the active metabolite can be removed by haemodialysis.

Shelf life

36 months.

Storage Condition

Store at temperature below 30°C.

Nature and contents of container

Blister pack of 30's (3 x 10's) and 100's (10 x 10's).

Manufactured by:


AUROBINDO
Aurobindo Pharma Ltd,
Unit III, Survey No. 313 & 314,
Bachupally, Bachupally Mandal,
Medchal-Malkajgiri District,
Telangana State, India.

Regd. Office: Plot No.: 2, Maitrivihar,
Ameerpet, Hyderabad-500 038,
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Product Registration Holder in Malaysia:

Healol Pharmaceuticals Sdn Bhd, 74-3,
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53300 Kuala Lumpur, Malaysia.

DATE OF PREPARATION OF THIS LEAFLET

December 2024